

Clinical Features

Large bullae occur infrequently in dermatitis herpetiformis (DH). Vesicles, particularly when located on the palms, may be hemorrhagic. The recurrent appearance and resolution of lesions can lead to areas of hyperpigmentation and hypopigmentation. Some patients present with only crusted lesions, and a careful examination may fail to identify a primary lesion. A herpetiform (herpes-like) grouping of lesions is often observed in certain areas, although many lesions may appear as individual, non-grouped lesions.

Symptoms vary widely: most patients experience significant burning and itching, while a rare individual may have minimal or no symptoms. Many patients can anticipate the onset of a new lesion 8 to 12 hours in advance due to localized stinging, burning, or itching at the site.

The characteristic symmetric distribution typically involves the elbows, knees, buttocks, shoulders, and sacral area (see Figures 59-1 to 59-4). Although these sites are most commonly affected, lesions frequently occur on the scalp and posterior nuchal region. The face and facial hairline are also commonly involved. Mucous membrane lesions are uncommon, as are lesions on the palms and soles.

Laboratory Tests

In Vivo–Bound Immunoglobulin A and Complement

Following Cormane's discovery that both perilesional and clinically uninvolved skin in DH patients show granular (or fibrillar) immunoglobulin (Ig) deposits in the dermal papillary tips, van der Meer demonstrated that IgA is the most consistently detected immunoglobulin class in the skin of these patients (Figure 59-5). Notably, IgA deposits are not found in the skin of patients with isolated gluten-sensitive enteropathy (celiac disease) without DH.

The presence of granular IgA deposits in normal-appearing skin is the most reliable diagnostic criterion for DH. These deposits are unaffected by

pharmacologic treatment but may diminish in intensity or resolve with long-term adherence to a gluten-free diet. The distribution of IgA deposits is not uniform across the skin and is often more readily detected in normal-appearing skin adjacent to active lesions.

In addition to IgA, other immunoglobulins may occasionally be bound in the same dermal papillary regions in DH. However, IgA deposits can also be observed in other conditions such as bullous pemphigoid, cicatricial pemphigoid, Henoch-Schönlein purpura, and alcoholic liver disease—though the patterns of deposition differ from those seen in DH.